



OFFICIAL

NMHS POLICY

Family and Domestic Violence (Identifying and Responding to)

Scope (Audience)	All NMHS workers
Scope (Area)	All areas of NMHS which provide services to adults
Summary	This Policy outlines the minimum requirements for NMHS sites and services when identifying (including routine screening) and responding to patients experiencing family and domestic violence, in accordance with relevant legislative and mandatory requirements. This policy does not address responding to staff disclosures of family and domestic violence.
Risk Rating	Medium
Policy Set	Clinical Planning
Policy Sponsor	Executive Director, Strategy and Transformation
Policy Contact	Manager, Women's Health Strategy and Programs
Version	1.0
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COMPLIANCE WITH THIS DOCUMENT IS MANDATORY

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1. Aim

This Policy outlines the minimum requirements for NMHS workers when identifying and responding to patients experiencing family and domestic violence (FDV), in accordance with the following Western Australian legislative and mandatory requirements:

- [Children and Community Services Act 2004](#)
- [Restraining Orders Act 1997](#)
- [Health Services Act 2016](#)
- [Health Services \(Information\) Regulations 2017](#)
- [Privacy and Responsible Information Sharing Act 2024](#)

This policy is to be read in conjunction with [NMHS Family and Domestic Violence Framework 2021 – 2026](#) and further guidance provided under the [Guidelines for Protecting Children 2020](#).

This policy **does not** address responding to staff disclosures of FDV. For more information on responding to staff disclosures, refer to the [NMHS FDV Hub page](#).

2. Background

FDV can be experienced by anyone, regardless of their gender or any other demographic factor. However, there are some priority population groups ([refer to NMHS FDV Framework](#)) that experience FDV and/or are significantly impacted by their experience of FDV at much higher rates. These include women and LGBTQIA+ people.

This policy uses gendered language in line with the [WA Department of Communities Path to Safety Strategy for 2020 – 2030](#), highlighting that FDV is a significant form of gender-based violence prevalent in society today.

It is not the intention of this policy to exclude any patient(s) or population groups. NMHS workers must respond to all disclosures of FDV in a trauma-informed manner, to ensure the safety, health and wellbeing of all patients.

3. Policy requirements

All NMHS sites and services must develop and implement local procedures or guidelines addressing the following minimum requirements:

- Outlining local processes on the management of FDV, including:
 - Identifying FDV ([section 3.1](#)),
 - Responding to FDV ([section 3.2](#)) and
 - Documentation and record keeping ([section 3.3](#)).
- Outlining roles and responsibilities for key workers identifying and responding to FDV.

3.1 Identifying FDV

FDV can be identified by either observing indicators of abuse during planned or unplanned presentations, during routine FDV screening, and/or through patient disclosure.

FDV screening and/or enquiry must be carried out:

- in a safe and private location.
- with the patient alone, unless the patient presents with a child or children under 2 years of age.
- by an experienced clinician who has completed essential FDV training (see [section 3.6](#)).

3.1.1 Indicators of FDV

Risk factors and signs of FDV are evidence-based indicators that help identify the likelihood or severity of FDV. Recognising these indicators, either through observation or direct enquiry, is key to assessing risk and taking appropriate action. Clinicians must be familiar with the signs and risk factors (Refer to [Appendices 1](#) and [2](#)).

If signs of FDV or risk factors are identified, clinicians are to enquire about FDV, unless Social Work is involved. If Social Work is already involved, clinicians are to consult with Social Work prior to making further enquiry. Enquiry can be conducted using a screening tool and/or asking the patient directly. Enquiries are to be guided by principles of trauma-informed and person-centred care.

3.1.2 Routine FDV screening

FDV screening is required in some clinical settings as patients accessing these services are at increased risk and will benefit the most from screening.

Screening does not assume that an individual is subject to FDV but provides people from priority population groups ([refer to NMHS FDV Framework](#)) with an opportunity for disclosure.

Clinicians are to screen patients in the following settings:

- Mental health services
- Gynaecology services
- Obstetric and maternity services - Obstetrics patients are to be screened a minimum of three times:
 - at initial antenatal visit
 - in the third trimester (Note: Screening not required if there is already Social Work involvement)
 - postnatally, prior to discharge from hospital (Note: Screening not required if there is already Social Work involvement)
 - unbooked obstetric patients at their first contact with the hospital.
- Where an FDV risk has been identified by another professional (e.g. ambulance services, general practitioner, community allied health).

Screening must be performed utilising an appropriate screening tool. NMHS sites and services must determine and specify which screening tools are appropriate for use to suit local context. Screening tools must be piloted before local implementation and evaluated regularly to ensure continued appropriateness. Available screening tools include:

- Screening for Family and Domestic Violence (FDV950) form
- Risk Assessment and Management Plan (RAMP) form
- Common Risk Assessment and Risk Management Framework (CRARMF) screening tool ([WA Department of Communities form](#))

3.2 Responding to FDV

If FDV is disclosed or otherwise indicated, clinicians **must**:

- determine if there is a serious and **immediate risk** of harm to the patient, any child, NMHS workers or others. **If so, call WA Police on 000**. If the risk is serious, but not immediate, you may report/make a referral to WA Police (131 444);

- follow local response processes as outlined in site-specific procedure or guideline relating to escalation, referral and support (see section [3.2.1](#)), crisis responses and safety planning;
- follow the [WA Health Procedure for Responding to a Recent Sexual Assault](#), if a sexual assault has occurred or is suspected;
- comply with mandatory reporting requirements (see [section 3.4.1](#)); and
- comply with requirements relating to information collection, access, use and disclosure (see [section 3.5](#)).

Victim-survivors must not be blamed or judged if they prefer not to leave their abusive situations, as they are the experts in their safety. Person-centred, culturally appropriate response pathways will be made available for all victim-survivors and victim-survivors engaged with a NMHS service.

3.2.1 Referral and support

Site-specific procedures or guidelines must clearly outline local referral and support pathways, meeting the following minimum requirements:

- referral to a social worker or other qualified clinician to conduct a comprehensive risk assessment and safety planning;
- if a social worker is unavailable, referral to appropriate support services (e.g. WA Police, WA Department of Communities Crisis Care, Domestic Violence Helpline, local FDV support services). The Assessment for Family and Domestic Violence (FDV951) form and the [Common Risk Assessment and Risk Management Framework](#) are available to guide clinicians in making judgements on level of risk.

Referrals must include relevant background information and risk, including (but not limited to):

- patient's concerns for their own safety
- concerns for children's safety (if applicable)
- availability of safe accommodation options
- willingness to engage with trusted person or police if unsafe
- identified risk factors (refer to [Appendix 2](#) to help determine risk).

Note: Referrals are more effective when a clinician initiates the process on behalf of the patient, with their informed consent. Where possible, contact the service when the patient is present. In the absence of informed consent, a referral would need to be permitted under the [Health Services Act 2016](#). Refer to section [3.5.3 consent to share information](#).

3.2.2 Language assistance

Language assistance may be required for patients who have limited English proficiency or for those who are deaf or hard of hearing. Refer to the [WA Health Language Services Policy \(MP 0051/17\)](#) and the [WA Health System Language Services Guidelines](#) for guidance.

When arranging an interpreter in the context of FDV, clinicians are to consult with the patient about their preference for the interpreter's gender and any potential connections the interpreter may have with the local community, as these factors can impact the patient's safety and confidentiality. Interpreters engaged over the telephone may be the preferred option for patients experiencing FDV, to protect anonymity and maintain safety.

3.2.3 Cultural Considerations

- **Aboriginal people**

NMHS is committed to working in partnership with Aboriginal people to ensure access to culturally appropriate care and support. Clinicians will strive to meet the needs of Aboriginal FDV victim-survivors by identifying and responding to their cultural needs, working within a holistic framework that recognises the importance of connection to country, culture, spirituality, family and community.

Aboriginal patients are to be offered the support of an Aboriginal Liaison Officer or an Aboriginal Health Worker to assist with cultural sensitivity. Aboriginal Interpreter Services can be accessed to facilitate communication (refer to section [3.2.2 Language assistance](#)).

- **Women from Culturally and Linguistically Diverse (CaLD) backgrounds**

Women from CaLD backgrounds and communities may hold diverse social, cultural, spiritual and emotional perspectives on FDV. Clinicians responding to FDV experiences of patients from CaLD backgrounds require a basic understanding of the different circumstances of FDV experiences as well as the different risk and protective factors for this priority population group. NMHS provides specialised training for all health workers in this area, including for social workers. Information on training is available at [King Edward Memorial Hospital - Women's Health Strategy and Programs education and training](#).

3.2.4 Patients with impaired decision-making capacity

If it is determined that the patient does not have decision making capacity, and the suspected perpetrator is their guardian, enduring guardian, or next of kin, urgent legal intervention may be required. A State Administrative Tribunal (SAT) application or other legal intervention may be necessary to appoint an alternative substitute decision-maker, however the least restrictive option(s) must be considered. Consultation with Social Work is recommended. Refer to the [WA Health Mandatory Policy Consent to Treatment Policy \(MP 0175/22\)](#) and the [WA Hierarchy of Treatment Decision-Makers](#) for further guidance.

3.2.5 Restraining orders or protective bail condition

NMHS workers are to consider whether a patient is protected by a family violence restraining order or protective bail conditions. Where a family violence restraining order, protective bail conditions, or another similar order is in force, NMHS workers should consult with the local Social Work team and/or their line manager.

3.3 Documentation and record keeping

All workers must comply with the [NMHS Clinical Documentation Policy](#). Additionally, clinicians must:

- Maintain clear, accurate, objective, thorough, and timely documentation of FDV related disclosures, assessments, injuries, observations, planning, and decision-making in the patient's healthcare record (Note: **Do not** document FDV in records that are patient held e.g. hand-held maternity records). This includes, but is not limited to, written notes, images, videos, and security footage, as appropriate, which describes:
 - the perpetrator's behaviours and actions, and their impact on the victim-survivor
 - actions conducted by the victim-survivor to increase their safety

- while ensuring the perpetrator's accountability is important, the safety of the victim-survivor is paramount and is always the top priority, including the safety of any children in the care of the victim-survivor.
- Avoid victim-blaming or mutualising language.
- Always isolate the victim-survivor from the perpetrator when documenting their experience and/or any safety planning.

Note: a patient's healthcare records may become evidence in future legal proceedings, including but not limited to criminal, family and children's court proceedings.

3.4 Reporting requirements

3.4.1 Mandatory reporting of child sexual abuse

Within the WA health system, doctors, nurses, midwives and psychologists are mandatory reporters of child sexual abuse.

Section 124B of the [Children and Community Services Act 2004 \(WA\)](#) places a mandatory duty on specified health professionals to report to the Department of Communities if they form a belief on reasonable grounds in the course of their paid or unpaid work that a child:

- has been the subject of sexual abuse that occurred on or after commencement day (commencement day is 1 January 2009 for doctors, nurses and midwives, and 1 May 2024 for psychologists); or
- is the subject of ongoing sexual abuse.

Under s 124B(1), a mandatory report must be made as soon as practicable after the reporter forms their belief. This is important as the earlier a report is received, the earlier steps can be taken to protect a child, where this is necessary.

Failure to make a mandatory report is an offence, unless there is a valid defence under s 124B(3).

Current procedural guidance and support for mandatory reporters can be accessed via the Department of Communities [Mandatory Reporting of Child Sexual Abuse in WA - Resources](#) webpage.

3.4.2 Reporting child abuse

Under s 28(2)(c) of the [Children and Community Services Act 2004 \(WA\)](#), a child is considered to be in need of protection if the child has suffered, or is likely to suffer, harm as a result of one or more of the following:

- physical abuse
- sexual abuse
- emotional abuse (including exposure to FDV)
- neglect (including medical neglect).

A child is exposed to FDV if the child sees or hears the violence or otherwise experiences the effects of the violence (s 3 of the [Children and Community Services Act 2004 \(WA\)](#) and s 6A of the [Restraining Orders Act 1997 \(WA\)](#)).

Any infant or child exposed to FDV is to be considered at risk of harm and referred appropriately to Social Work for assessment. If Social Work is not available and the risk of harm is serious, a report may be made by another suitable clinician to the Department

of Communities. It is best practice to seek patient consent, however a report can be made without patient consent, if authorised under s 220(1) of the [Health Services Act 2016 \(WA\)](#).

3.5 Information collection, access, use, and disclosure relating FDV

All workers must adhere to the [NMHS Healthcare Record Access, Use and Disclosure Policy](#).

Clinicians must ensure a patient's healthcare records cannot be accessed by the perpetrator - refer to [section 3.3 Documentation and Record Keeping](#). Local procedures or guidelines must include guidance on protecting confidentiality of patient records. In situations where a perpetrator has access to the patient's healthcare records, contact the Information Custodian to explore options to restrict access. In situations where a perpetrator has accessed the patient's healthcare record, clinicians must report it as an information breach in accordance with the [NMHS Information Breach Response Procedure](#).

3.5.1 Sharing information under the Children and Community Services Act 2004

Only authorised positions, as documented in the [NMHS Authorisation, Delegation and Decision-Making Schedule](#), can disclose **relevant information** to a prescribed authority, non-government provider or the Department of Communities in response to a request made under Part 3 Division 6 of the [Children and Community Services Act 2004 \(WA\)](#).

Note: The information sharing provisions in the [Children and Community Services Act 2004 \(WA\)](#) **do not compel** an agency to share information, and the decision to share must be based on sound professional judgement. A decision not to share information must not compromise a person's safety or wellbeing. Refer to [Interagency Information Sharing for High Risk Cases](#).

3.5.2 Sharing information under the Health Services Act 2016

Under s 219 of the [Health Services Act 2016 \(WA\)](#), it is an offence for a person to collect, use or disclose information obtained by the person because of their office, position, employment or engagement, unless authorised under s 220(1).

Section 220(1)(i) of the [Health Services Act 2016 \(WA\)](#) and reg 5(1)(a) and (b) of the [Health Services \(Information\) Regulations 2017 \(WA\)](#) provide that the collection, use or disclosure of information is authorised where the collection, use or disclosure is reasonably necessary to lessen or prevent:

- a serious risk to the life, health or safety of any individual: or
- a real or immediate risk of danger to the public.

Under s 217 of the [Health Services Act 2016 \(WA\)](#), the appointed Information Custodian or Information Steward is authorised to disclose relevant information about a patient of the health service provider to any person who has a sufficient interest in the treatment, care, health, safety or wellbeing of the patient (refer to IM 01.01 in the [NMHS Authorisations, Delegations and Decision-Making Schedule](#)).

3.5.3 Consent to share information

Information can be shared with or without the person's consent if an exception under s 220(1) of the [Health Services Act 2016 \(WA\)](#) applies. Obtaining consent is preferred unless there is a good reason not to do so, such as in the following circumstances:

- An adult or child is at serious risk of harm or there is a real or immediate risk of danger to the public.
- Seeking consent from the adult victim will likely place the adult and/or child victim at risk of further harm.
- Information is being disclosed for the purposes of the investigation of a suspected offence.
- Reasonable efforts to obtain consent have failed; or
- It is clear from previous contact that consent would not be given.

Where consent is obtained, it can be given verbally or in writing and must be documented in the person's healthcare record. Use an interpreter if needed.

3.5.4 Disclosure required by law

Disclosure of information may be required by law in some circumstances, including:

- Under the order of a court or other person or body acting judicially; such as in compliance with a subpoena. Legal advice may be sought before producing any FDV information under a subpoena, particularly where records include a private communication shared with a counsellor or with another clinician who assists victims of sexual assault or family violence.
- When an Order to Produce or Notice to Produce is issued and served on NMHS.

3.6 Training and support

NMHS supports clinicians in identifying and responding to FDV by providing access to essential online learning, face to face training and resources. Refer to the [NMHS Mandatory Training Policy](#) and site specific procedures or guidelines for essential training requirements. FDV Training is available to all NMHS workers via [King Edward Memorial Hospital - Women's Health Strategy and Programs education and training](#).

4. Roles and Responsibilities

Roles	Responsibilities
Executive Directors	Are responsible for ensuring implementation of this Policy across their accountable areas.
Directors/Heads of Department	Are responsible for: • Implementing this Policy within their area and/or department. • Ensuring the development of a local procedure or guideline addressing the principles outlined in this Policy. • Reporting any implementation issues to this Policy author and escalating to sponsor as appropriate.
Senior staff and managers	Are responsible for: • Monitoring compliance with this Policy. • Monitoring compliance with FDV essential training. • Investigating clinical incidents where FDV is a causal factor. • As a delegated authority (Tier 6 or above), determining release of information to external organisation, if required. • Providing leadership and consultation with worker for FDV high-risk presentations.

	- Supporting and/or facilitating responses to children at risk of abuse and neglect, including reporting to Department of Communities. - Escalating issues as per local staffing and governance structures.
Clinicians	Are responsible for: - Complying with this Policy. - Completing FDV essential training. - Maintaining familiarity of observable signs and risk factors of FDV. - Performing routine screening and targeted enquiry where defined. - Following site/area specific procedure or guidelines to appropriately support and refer patients experiencing FDV. - Complying with reporting requirements. - Documenting appropriately as outlined.
Information Custodians	Are responsible for ensuring access, use and disclosure of FDV related information contained within their information asset(s) complies with the Health Services Act 2016, the Health Services (Information) Regulations 2017, the WA Health Information Management Policy Framework and the NMHS Healthcare Record Access, Use and Disclosure Policy.
All NMHS workers	Are responsible for: - Having knowledge of the policy documents relevant to their roles and how to access these documents. - Complying with mandatory requirements outlined in these documents. - Reporting any potential hazards, including psychosocial (such as vicarious trauma), as per the NMHS Hazard/Incident Reporting and Investigation Policy. - Assisting with policy development/reviews wherever required. - Reporting to their immediate line manager any potential non-compliance or implementation issues.

5. Monitoring and Evaluation

Compliance with this Policy is mandatory. There are legislative obligations relating to this Policy where a statutory penalty may apply for non-compliance. **Note:** to support the implementation of this policy, a grace period of 12 months is applied before mandatory compliance is required.

All sites and services within NMHS have a responsibility to monitor local adherence to the principles and requirements of this Policy. A compliance and evaluation audit will be periodically undertaken by each NMHS site and service conducting FDV screening and response activities to ensure alignment with these Policy requirements.

This Policy will be evaluated as required to determine its currency, relevance, effectiveness, efficiency, and sustainability. At a minimum, it will be reviewed **every three years** using a risk management approach in accordance with the [NMHS Policy Governance Framework](#).

6. Definitions

Terms	Definitions
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Adult	A person who has reached 18 years of age.
Child abuse	All forms of physical and/or emotional ill-treatment, sexual abuse, neglect or negligent treatment or commercial or other exploitation, resulting in actual or potential harm to the child's health, survival, development or dignity in the context of a relationship of responsibility, trust or power. There are four main types of child abuse: ○ physical ○ sexual ○ emotional – including psychological abuse and exposure to family and domestic violence (FDV) ○ neglect.
Clinician	In this document, the term 'clinician' refers to all healthcare providers who deliver direct clinical care to patients. This includes, but is not limited to, doctors, dental officers, nurses, midwives, pharmacists, nurse practitioners, Aboriginal health workers or practitioners, paramedics, social workers, and allied health practitioners.
Coercive control	A pattern of controlling behaviour, used by a perpetrator to establish and maintain control over another person. Coercive control is almost always an underlying dynamic of FDV and intimate partner violence. Coercive control can involve subtle or covert behaviours that would be perceived as innocuous to an external observer but would be experienced as abusive or controlling by the victim-survivor.
Elder abuse	A single or repeated act or lack of appropriate action, occurring within any relationship where there is an expectation of trust which causes harm or distress to an older person. Elder abuse can take various forms, including physical, psychological, financial and sexual abuse, as well as neglect.
Family and domestic violence	The overarching term for an ongoing pattern of behaviours intended to coerce, control or create fear within a family or intimate relationship. This includes physical harm or threats of physical harm, financial, emotional and psychological abuse, sexual violence or any other behaviour which causes the victim-survivor to live in fear
Family violence	Preferred term by Aboriginal people, noting the ways violence can manifest across extended family networks.
Identification	Refers to the recognition of signs and/or risk factors of FDV observed during a clinical encounter or other assessment.
Perpetrator	Used to refer to people who have used or are using violence in their relationships. The use of this terminology reflects the importance of holding the person to account for their choice to use violent behaviour and inflict harm. It is not intended to reflect a person's identity or capacity for change
Relevant information	For the purposes of s 23 of the <i>Children and Community Services Act 2004</i>, 'relevant information' means – (a) Information that is, or is likely to be, relevant to –

	(i) The wellbeing of a child or a class or group of children; or (ii) The safety of a person who has been subjected to, or exposed to, family violence; or (b) other information of a kind prescribed by the regulations. For the purposes of s 28A of the Children and Community Services Act 2004, 'relevant information' means – (a) information that is, or is likely to be, relevant to: (i) the wellbeing of a child or a class or group of children; (ii) the safety of a person who has been subjected to, or exposed to, family violence; or (b) other information of a kind prescribed by the regulations. For the purposes of s 217 of the Health Services Act 2016, 'relevant information' means health information that in the opinion of the chief executive of a health service provider, is or is likely to be relevant to any of the following – (a) the treatment or care of a patient who has been, is being, or will or may be, provided with a health service by the health service provider; (b) the health, safety or wellbeing of a patient who has been, is being, or will or may be, provided with a health service by the health service provider.
Screening	The use of questions to explore the possibility of FDV being present, due to concerns through observation or other assessment.
Trauma-informed care	Involves understanding how trauma shapes a person's world view and functioning. Trauma-informed care will incorporate principles of safety, trust, collaboration and choice in service delivery.
Victim-survivor	The term is used to refer to people who have previously been or are currently being abused by a family member or intimate partner. This includes survivors and victim-survivors. The term aims to acknowledge the harm caused by FDV and in no way reflects the person's full identity.
Worker	According to the Work Health and Safety Act 2020 (WA), a person is a worker if the person carries out work in any capacity for NMHS, including work as – (a) an employee; or (b) a contractor or subcontractor; or (c) an employee of a contractor or subcontractor; or (d) an employee of a labour hire company who has been assigned to work in NMHS; or (e) an outworker; or (f) an apprentice or trainee; or (g) a student gaining work experience; or (h) a volunteer; or (i) a person of a prescribed class

7. References

Department of Communities, Western Australian Government. *Western Australian Family and Domestic Violence Common Risk Assessment and Risk Management Framework* (2nd ed.). <https://www.wa.gov.au/system/files/2021-10/CRARMF.pdf>

Government of Western Australia. *Family and Domestic Violence Framework 2021 – 2026*. Perth: North Metropolitan Health Service. <https://www.nmhs.health.wa.gov.au/~media/HSPs/NMHS/Hospitals/WNHS/Documents/Professionals/FDV/FDV-Framework.pdf>

Government of Western Australia. *Path to Safety: Western Australia's Strategy to Reduce Family and Domestic Violence 2020–2030*. Perth: Department of Communities, 2020. <https://www.wa.gov.au/system/files/2021-04/fdv-strategy-2020-2030.pdf>.

The Royal Australian College of General Practitioners. Abuse and violence: working with our patients in general practice, 5th edition (the White Book). <https://www.racgp.org.au/getattachment/4ab6102c-67d9-4440-9398-a3ae759164ef/Abuse-and-violence-Working-with-our-patients-in-general-practice.aspx>

Toivonen, C., & Backhouse, C. (2018). National Risk Assessment Principles for domestic and family violence: Quick reference guide for practitioners (ANROWS Insights 10/2018). Sydney, NSW: ANROWS

Victoria State Government. MARAM Practice guides, Responsibility 2: identification of family violence risk; [Responsibility 2: Identification of family violence risk | vic.gov.au](#)

8. Related Documents

Legislation	• Children and Community Services Act 2004 (WA) • Health Services (Information) Regulations 2017 (WA) • Health Services Act 2016 (WA) • Restraining Orders Act 1997 (WA) • Privacy and Responsible Information Sharing Act 2024 (WA) • Work Health and Safety Act 2020 (WA)
Government policies	• Family and Domestic Violence, Paid Leave and Workplace Support - Premier's Circular 2025/07
WA Health mandatory policies	• WA Health Information Management Policy Framework • WA Health Consent to Treatment Policy (MP 0175/22) and WA Hierarchy of Treatment Decision-Makers • WA Health Language Services Policy (MP 0051/17) and WA Health System Language Services Guidelines • WA Health Procedure for Responding to a Recent Sexual Assault
NMHS Policy documents	• NMHS Clinical Documentation Policy • NMHS Family and Domestic Violence Leave Procedure • NMHS Hazard/Incident Reporting and Investigation • NMHS Healthcare Record Access, Use and Disclosure Policy • NMHS Information Breach Response Procedure

	• NMHS Mandatory Training Policy
Standards	• NSQHS Standards – Comprehensive care standard
Other	• WA Health Guidelines for protecting Children 2020

9. Resources

- [Legal Aid WA: Family violence restraining orders](#)
- [Mandatory Reporting of Child Sexual Abuse in WA - Resources](#)
- [WNHS Sexual Assault Resources Centre Hub](#)
- [WNHS Family and Domestic Violence Screening Tool In 20 Languages](#)
- [Western Australian Family and Domestic Violence Common Risk Assessment and Risk Management Framework](#)
- [NMHS Workplace hazards](#)

10. Recognition of victims and survivors

NMHS recognises the strength and resilience of adults, children, and young people who have experienced domestic, family and sexual violence and acknowledge that it is essential that responses to domestic, family, and sexual violence are informed by their lived experience, expert knowledge and advocacy.

11. Document Control

Version	Policy author	Summary of changes	Document Reference	Approval date
1.0	T. Wisby	Original version	D/25/301765	24/11/2025

This document can be made available in alternative formats on request for a person with a disability.

Within Western Australia, the term Aboriginal is used in preference to Aboriginal and Torres Strait Islander, in recognition that Aboriginal people are the original inhabitants of Western Australian. Aboriginal and Torres Strait Islander may be referred to in the national context and Indigenous may be referred to in the international context. No disrespect is intended to our Torre Strait Island colleagues and community.

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Appendix 1 Observable signs that may indicate FDV

Table 1: Signs that may indicate FDV in adult victim-survivor^{1,2,3}

	Signs that may indicate FDV is occurring in an adult victim-survivor	
Physical/ Sexual	• obvious injuries (bruises, fractures, scars or cuts). • 'Accident prone' with a history of injury, untreated injuries and multiple injuries, especially in various stages of healing • strangulation • fatigue • gastrointestinal disorders	• sexual assault / reproductive control • sexually transmitted infections • terminations of pregnancy • pregnancy complications • miscarriage and stillbirth • nausea • change in appetite • chronic pelvic pain • chronic abdominal pain
Psychological	• depression • anxiety • eating disorders • somatic disorders • confusion • memory issues • nightmares • decision making difficulties • self-doubt/ hypercriticism of self	• insomnia/sleep problems • impaired concentration • harmful alcohol use • licit and illicit drug abuse • physical exhaustion • suicidal ideation • self-harming behaviour • unexplained paranoia
Emotional	• fear • shame • anger • irritability • stress • feeling of overwhelm	• hyper-alertness and hypervigilance • feelings of worthlessness and hopelessness • feeling disassociated and emotionally numb
Social/ financial	• homelessness • unemployment • financial debt • inability to access funds • inability to afford basics	• isolation • no friend or family support • parenting difficulties • signs of neglect (poor hygiene, malnourishment, missing aids etc)
Demeanour	• unconvincing explanations of injuries • delay in seeking treatment for injuries • describes partner/relative as controlling. • anxiety in the presence of a partner/relative	• accompanied by partner/relative, who pushes to speak for the patient • feeling stressed about needing to be home by a certain time. • reluctance to talk openly • reluctance to follow advice

*Note this is not an exhaustive list and clinicians must use their professional judgment to identify if FDV risk is present.

¹ Victoria State Government. MARAM Practice guides, Responsibility 2: identification of family violence risk.

² The Royal Australian College of General Practitioners. Abuse and violence: working with our patients in general practice, 5th edition (the White Book).

³ Department of Communities, Western Australian Government. Western Australian Family and Domestic Violence Common Risk Assessment and Risk Management Framework (2nd ed.), Accessed July 25, 2025, <https://www.wa.gov.au/system/files/2021-10/CRARMF.pdf>

Appendix 2 High risk factors for FDV

Table 2 Risk factors associated with a higher likelihood of FDV reoccurring, serious injury or death^{2,3,4,5}

High Risk Factor	Information
History of FDV	Psychological and emotional abuse has been found to be a good predictor of continued abuse, including physical abuse. Previous physical assaults also predict future assaults.
Separation (actual or pending)	For those who are experiencing FDV, the high-risk periods include immediately prior to taking action, and during the initial stages of, or immediately after, separation. Children are also at increased risk of harm.
Intimate partner sexual violence	Research has shown that intimate partner sexual violence was the strongest indicator of escalating frequency and severity of violence, more so than stalking, strangulation and abuse during pregnancy
Non-fatal strangulation (NFS) (or choking)	NFS is a key marker for the escalation of violence in a domestic relationship, and a strong indicator of future risk for serious harm and death of the victim-survivor.
Stalking	Stalking, when coupled with physical assault, is strongly connected to murder or attempted murder. Stalking behaviours include technology surveillance, GPS tracking, persistent phoning/texting, and contact against court order conditions.
Threats to kill victim-survivor	Evidence suggests that a perpetrator's threat to kill a person is often genuine. Attempted strangulation should be considered an attempt to kill.
Harm, or threats to harm/kill pets or other animals	Cruelty and harm directed to pets and other animals can indicate risk of future or more severe violence.
Perpetrators access to, or use of, weapons	Use of a weapon (any tool used by the perpetrator that could injure, kill or destroy property) indicates high risk, particularly if used in the most recent violent incident, as past behaviour strongly predicts future behaviour.
Escalation – increase in severity and/or frequency of violence	Violence occurring more often or becoming worse has been found to be associated with lethal outcomes for victims.
Coercive control	Coercive control is a pattern of abusive behaviour that often underpins FDV. Coercive control can heighten the risk of lethality, particularly in contexts where other high-risk factors are present, such as attempts by the victim-survivor to leave the relationship.
Pregnancy and birth	Violence against a pregnant person by a partner is a significant indicator of future harm to the woman and child and is the primary

⁴ Toivonen, C., & Backhouse, C. National Risk Assessment Principles for domestic and family violence: Quick reference guide for practitioners (ANROWS Insights 10/2018). Sydney, NSW: ANROWS

⁵ Sharman, L. S., Douglas, H., & Fitzgerald, R. 2021. Review of domestic violence deaths involving fatal or non-fatal strangulation in Queensland. (The University of Melbourne/The University of Queensland).

	cause of death to mothers during pregnancy. Violence often begins during pregnancy and, when previously occurring, often escalates in frequency and severity.
Victim-survivor's self-perception of risk	Whilst a person's perception of their own risk of experiencing future violence is not always sufficient by itself to accurately determine severity or incidence of violence, it is important to consider. A person's own assessment of their level of risk can also provide information in relation to their safety management.
Suicide threats and attempts	Threats of suicide, like most threats in the context of FDV, are a strategy used by perpetrators to exert control. Threats or attempts at suicide have been found to be a risk factor for murder-suicide.
Court orders and parenting proceedings	FDV often escalates among separating parents. The perpetrator may use their joint parenting role or judicial options as a way of exercising control over the victim-survivor.
Misuse of drugs or excessive alcohol consumption	Alcohol and other drug use can be a risk factor or coping mechanism for FDV and has been associated with both perpetration and victimisation and may precede or follow violence.
Financial difficulties	Low income and financial stress, may be associated with increased risk for victim-survivors of FDV.
Isolation	Isolation, including limiting interactions with family, friends, social supports, and community support programs is a control tactic and increases the risk of harm.
Family conflict	Abuse can be a continuation of FDV that re-emerges as abuse in the caring situation.
Dependency	A dependency on others, including assistance for daily living, social, emotional, physical, financial and spiritual support, increases risk of abuse. This dependency may hinder the person leaving the abusive situation or reporting the situation.
Cognitive impairment or other disability	Cognitive impairment and other forms of disability have a strong association with higher risk of FDV. If a victim-survivor has dementia, they are more at risk of experiencing financial abuse.
Carer stress	FDV risk may increase when caregivers experience significant burden or stress, particularly in the presence of mental health issues, alcohol or substance use, financial dependency, or a history of trauma or abuse.